

DPSH MUN 2026

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**UNITED NATIONS
COMMISSION ON
NARCOTICS AND DRUGS**



DPSH MUN 2026



AGENDA:

“Reframing Narcotics Policy: Distinguishing Addiction from Crime and Examining Capital Punishment.”

*Reframing Narcotics Policy: Distinguishing Addiction from Crime and Examining
Capital Punishment*

Background Paper for UNCND DPSHMUN 26'

Introduction

The agenda, "Reframing Narcotics Policy: Distinguishing Addiction from Crime and Examining Capital Punishment," is an opportunity for delegates to confront a central tension in contemporary drug governance. Narcotics policy has long treated drugs as a threat to public order, national security and social discipline. Yet scientific and public health evidence increasingly describes many forms of harmful drug use as a health condition or a disease shaped by dependence, trauma, poverty, mental illness, stigma and limited access to care. This distinction does not excuse violence, trafficking or organized criminality. It requires a more precise policy vocabulary: addiction should not be treated as crime, while exploitation, corruption and large-scale trafficking should remain serious criminal justice concerns.

The significance of the agenda lies in its practical and moral stakes. UNODC estimates that drug markets and drug-related harms remain substantial, while many people with drug use disorders do not receive treatment (UNODC, 2024). At the same time, criminal justice systems in many countries still process large numbers of people for use or possession. The death penalty adds a sharper controversy because several States retain capital punishment for drug offences, while UN human rights bodies maintain that capital punishment, where it exists, must be limited to the most serious crimes, understood as crimes of extreme gravity involving intentional killing (OHCHR, 2026). The intention of addressing this agenda is to effectively attempt to argue for a two-track approach: health-led responses for addiction and proportionate, rights-compliant enforcement against serious criminal markets, that being said this intention is an opinion that if you do choose to advocate for as a delegate it essentially means your country advocates for the same.

What am trying to say is, if your country DOES NOT advocate for this opinion, then DO NOT advocate for it as a delegate representing the country either (because then it would violate your country's foreign policy).

If you are a first timer reading this and did not understand the above paragraph on what the agenda is about then here are your following steps (Genuinely please ignore if you aren't a first timer, there will be relevant tips ahead specifically written for more experienced delegates).

1. To begin with, Understand What exactly is the UN? what are its roles and responsibilities, knowing a little history about how it all began helps, only surface level is required, please refrain from using or mentioning any of the history related to the roles of formation of the UN in your speeches (Gsl/mod/unmod – these are different types of debates/ or speech giving opportunities, read the delegate handbook for more detail on this).

2. Now, Research on what is the UNCND? You will come across UNODC as well, Brief information on these two bodies is already given in this document, read a little on both committees. It will help. Again, please refrain from using or mentioning any generic history on the roles or formation of UNCND or UNODC In your speeches (But if you are sure that you need to mention it for valid reasons you may do so).

3. Lastly Research about your country's involvement in UNCND before you start researching on the agenda itself (a good place to start is the Background guide which u are reading so if you have come this far, good job on that!) and after which you should research on your country's involvement/stand/actions/policy on the given agenda.

If all of this has gone over your head, three things-

□ A delegate is a diplomat who represents his country in a global setting.

An agenda is the issue/topic that is getting debated upon/ or discussed so that a (sort of) conclusive solution to the issue is found. (Keyword- Solution, The technical representation of solutions is called a draft resolution.)

- If you have not understood something, read it again, if you still don't understand it, ask someone who would know what it means. If you still don't understand, Ask the chair or the vice chair before the conference begins.

Now to continue the topic hand, here's a little history about the agenda...

Historical Evolution of Narcotics Policy: Global and National Contexts

Modern narcotics policy evolved from early twentieth-century efforts to regulate opium and other substances into a treaty-based system of international control. The core framework rests on the 1961 Single Convention on Narcotic Drugs as amended by the 1972 Protocol, the 1971 Convention on Psychotropic Substances and the 1988 Convention against Illicit Traffic in Narcotic Drugs and Psychotropic Substances. UNODC identifies these conventions as the principal legal instruments of international drug control, designed to ensure the availability of controlled substances for medical and scientific purposes while preventing diversion and illicit trafficking (UNODC, International Drug Control Conventions).

Although the treaties contain both health and enforcement objectives, national implementation often leaned toward policing, incarceration and punitive deterrence. Countries adopted different models depending on legal tradition, colonial history, religion, public health capacity, crime patterns and political culture. Some imposed severe penalties for possession and trafficking; others created administrative responses for personal use while preserving criminal penalties for supply. Recent cannabis regulation and decriminalization reforms show that the international debate is no longer a simple contest between prohibition and permissiveness. It is now a debate over evidence, proportionality, treaty flexibility and human rights.

Addiction as a Medical Condition: Scientific Basis, Public Health Approaches and

Evidence

WHO and UNODC describe drug use disorders as complex health conditions with biological, psychological, social and environmental determinants rather than merely a moral failure or a lack of discipline (WHO and UNODC, 2020). Dependence can involve craving, tolerance, withdrawal, relapse and impaired control over use, often interacting with trauma, exclusion, unemployment, untreated pain or mental illness. This medical framing does not eliminate personal responsibility, but it explains why punishment by itself rarely addresses the root causes of harmful drug use.

A public health response therefore prioritizes prevention, voluntary treatment, psychosocial support, medication-assisted treatment where appropriate, mental health care, harm reduction, aftercare and social reintegration. Harm reduction measures such as needle and syringe programmes, opioid agonist therapy, naloxone access, overdose prevention and HIV or hepatitis services aim to reduce death and disease among people who may not immediately stop using drugs. UNODCs World Drug Report 2024 frames the right to health in drug policy through availability, accessibility, acceptability, quality, non-discrimination and participation (UNODC, 2024). For delegates, the question is not whether health should replace all enforcement, but whether enforcement blocks treatment or drives vulnerable people further into risk.

Crime and Narcotics: The Criminalization Paradigm, Critiques and Alternative Models

The criminalization paradigm treats drug possession, consumption, production and trafficking primarily as offences against the State. Its defenders argue that law enforcement deters use, disrupts supply, protects communities and expresses social condemnation of dangerous conduct. These claims are strongest when applied to violence, coercion, large-scale trafficking, corruption, child exploitation and money laundering. Organized drug markets can weaken institutions and endanger communities, so the State has a legitimate duty to respond.

The difficulty is that broad criminalization often collapses very different situations into one category. A person dependent on opioids, a teenager possessing a small quantity, a coerced courier and a violent trafficker may all enter the criminal justice system even though their culpability and social risk differ markedly. UNODC reported that in 2022 roughly seven million people were in formal contact with police for drug offences, and about two-thirds of those contacts concerned drug use or possession for use (UNODC, 2024). Alternative models include decriminalization of small-scale possession, diversion to treatment, administrative penalties, proportional sentencing, regulated cannabis markets and hybrid systems that reserve criminal justice resources for serious trafficking and related violence.

The use of statistics and numbers are very common when discussing crime and the above-mentioned scenarios, it is important to remember that these numbers only make sense to be mentioned when there is intention and reason behind the statement or argument being made.

Capital Punishment in Narcotics Policy: Global Practices and Arguments for and Against

Capital punishment for drug offences is among the most controversial areas of narcotics policy. Harm Reduction International reported that 34 countries retained the death penalty for drug offences in law in 2024 and that at least 615 people were confirmed executed for such offences that year, excluding countries where secrecy prevents reliable documentation (HRI, 2025). Retentionist States usually justify this punishment through deterrence, incapacitation, retribution and sovereignty. They argue that drug trafficking destroys families, fuels dependence and threatens public security, and that severe penalties are necessary in vulnerable trafficking environments.

Opponents respond that the death penalty is irreversible, often discriminatory in practice and unsupported by conclusive evidence that it deters trafficking better than long imprisonment or targeted enforcement. They also argue that many people facing execution are low-level couriers, poor defendants, migrants or individuals with limited ability to expose organizers. Under contemporary human rights interpretation, drug offences do not normally meet the threshold of the most serious crimes because they do not involve intentional killing by the accused. The policy question is therefore not whether trafficking is serious, but whether execution is a necessary, proportionate and lawful response to it.

Diplomacy is the art of being able to discuss such delicate matters with sensitivity, patience and elegance. Lobbying and listening are two active strategies that can make or break your diplomatic efficacy as a delegate.

The modern meaning of the word debate is a structured, reasoned exchange of opposing ideas.

Please remember diplomats do not fight.

International Law and Human Rights: Treaties, Conventions, ICCPR and UN Positions

The international drug control conventions require States to prevent diversion, regulate controlled substances and act against illicit trafficking, but they do not require capital punishment. Their stated concern for the health and welfare of humankind leaves room for proportionate penalties, prevention, treatment and rehabilitation. The 2016 UNGASS outcome document, prepared through the Commission on Narcotic Drugs and adopted by the General Assembly, contains operational recommendations on demand reduction, supply reduction, access to controlled medicines, human rights, emerging challenges, international cooperation and development (UNODC, 2016).

Human rights law places independent limits on punishment. Article 6 of the ICCPR protects the right to life and allows the death penalty only in countries that have not abolished it, and only under strict safeguards. UN human rights mechanisms have interpreted the most serious crimes threshold as limited to crimes of extreme gravity involving intentional killing, excluding drug offences. OHCHR also argues that abolition is necessary for human dignity and that the death penalty lacks proof as an effective crime-control tool (OHCHR, Death Penalty). Thus, a State may enforce drug laws, but it must do so through legality, due process, proportionality, non-discrimination and respect for the right to health.

When you begin to learn and understand how to write draft resolutions, you will notice that existing official documents (Treaties, Conventions, Policy Protocols etc.) are often Recalled, Remembered, Reiterated, Reaffirmed, Recognized, or Noted as well.

Being able to put your solutions into a valid format and representation style is also a very important skill that can very easily set you apart from other delegates.

Case Studies: Portugal, Singapore, the Philippines and Other Models

Portugal is central to the reform debate because it decriminalized possession of all drugs for personal use in 2001 while retaining criminal penalties for trafficking. Possession became an administrative matter handled through Commissions for the Dissuasion of Drug Addiction, which combine legal, health and social perspectives. Reviews of the Portuguese model emphasize that the reform worked in combination with expanded treatment, harm reduction and social reintegration, and that the dramatic rise in use predicted by opponents did not materialize (Transform Drug Policy Foundation, 2021). Portugal therefore illustrates health- led decriminalization, not simple legalization.

Singapore represents a contrasting model. It retains strict drug laws and capital punishment for selected trafficking offences above statutory thresholds, although reforms have created limited discretion in certain courier cases. Its government argues that severity deters trafficking and protects society. Critics question whether the available evidence proves a unique deterrent effect and argue that mandatory or near-mandatory death sentencing raises concerns about proportionality, fair trial guarantees and the position of low-level couriers. Singapore is useful for debate because it presents the strongest sovereignty and deterrence argument for punitive policy.

The Philippines illustrates the dangers of treating drug control as a campaign of social cleansing. A 2020 OHCHR report found serious human rights violations linked to the anti-drug campaign, including widespread and systematic killings of alleged drug suspects and persistent impunity, with official figures then indicating at least 8,663 deaths and other estimates higher (OHCHR, 2020). Other jurisdictions offer additional lessons: some combine strong trafficking enforcement with treatment expansion; some regulate cannabis; and some, such as Malaysia and Pakistan, have moved to restrict mandatory capital punishment. These examples show that reform is a spectrum rather than a single template.

Ethical Considerations: Justice, Proportionality and Social Responsibility

Ethically, narcotics policy must balance personal responsibility, public safety and social responsibility.

A person who organizes trafficking, exploits minors, launders proceeds or uses violence may deserve serious punishment. A person living with dependence, homelessness or untreated trauma requires a different response. Justice is not achieved by treating unequal cases equally; it is achieved by recognizing differences in harm, culpability and capacity for rehabilitation.

Proportionality is especially important in the debate on capital punishment. Drug trafficking may be grave, but it usually does not involve intentional killing by the accused. Execution can also reproduce inequality when those punished most harshly are poor couriers while organizers remain protected. Social responsibility requires States to address the conditions that make harmful drug use and illicit markets grow: poverty, stigma, lack of treatment, weak institutions, conflict and exclusion. Humane policy does not deny crime; it insists that punishment alone cannot solve a problem rooted in health, economics and governance.

Ethics is a gray area, always has been and always will be, proceed with caution when you as a delegate question another country's ethics, you can only do that when the country that you represent is at least on face value ethical.

Debating on ethics is a more intermediate strategy in my experience and opinion. Its high-risk high reward, it can often backfire very easily.

Policy Alternatives: Harm Reduction, Decriminalization, Legalization and Hybrid

Approaches

Policy alternatives should be evaluated by outcomes: fewer deaths, reduced disease transmission, weaker organized crime power, improved treatment access, fairer sentencing, less corruption and safer communities. Harm reduction is often the most practical starting point because it reduces immediate risks without necessarily changing the legal status of drugs. Decriminalization goes further by removing criminal penalties for possession for personal use, thereby reducing stigma and allowing police to focus on serious crime. Legalization and regulation, most commonly for cannabis, may reduce illicit market profits but require strict controls on advertising, youth access, potency, taxation and commercialization.

A hybrid framework is likely to be most realistic for UNCND consensus because it allows States to adapt reforms to local conditions while preserving shared principles. It can acknowledge that Member States have different legal systems and security concerns while still setting a minimum standard of humanity and effectiveness. For example, a delegation may defend strict punishment for transnational trafficking while also supporting treatment access, diversion for personal possession and a moratorium on executions. This kind of compromise is valuable in UNCND because consensus usually depends on language that is principled but implementable. Such a framework could include the following measures:

- Remove the death penalty for drug offences and replace it with proportionate terms of imprisonment for serious trafficking, with judicial discretion and fair trial guarantees.
- Create diversion pathways for people arrested for personal possession, linking them to voluntary treatment, counselling, housing assistance or social services where appropriate.
- Expand evidence-based treatment, including opioid agonist therapy, psychosocial support, aftercare and confidentiality protections.

- Focus law enforcement on organized trafficking, corruption, money laundering, firearms, child exploitation and violent market actors rather than low-level possession. Collect
- transparent data on arrests, prosecutions, treatment access, overdose deaths, sentencing disparities and drug-related violence so policy can be evaluated rather than assumed.

This is my favourite part of the background guide because these are the sort of conclusions, I expect you all, both as a group (the committee as a whole) and as an individual (an individual who represents the given country as a whole) to be able to discuss, deliberate, debate and then agree or disagree upon (the voting).

This part involves drawing conclusive solutions based on everything every delegate has spoken and advocated for in the committee sessions, after which the committee is expected to write up draft resolutions, present them to the entire committee and lastly vote upon the presented draft resolution(s). There are nuances to this highly streamlined process which u may study for your own benefit and reference.

Conclusion before the conclusion

DPSHMUN is a conference held for two days which means two days of active debate and discussion, to be precise 490 minutes (which is roughly 8 hrs) which in my experience is not enough.

This means it is your responsibility as delegates to optimize the time you are given to the best of your potential, which in simple words means having solid preparation.

For first time MUNners, Understand the Rules of Procedure well, in the age of the internet you are a YouTube search away from “How to prepare for My First MUN”, followed by how to write speeches, how to debate and how to research.

For non-first timers, Go All Out.

Solid Research, when you are done with your country specific research with regards to the agenda, strategize and think of creative subtopics (topics that all delegates will want to speak on or can speak on due to the nature of the topic) that need to be discussed (moderated and unmoderated debate) in committee so that you can solve said issues.

REMEMBER you cannot add anything to the draft resolution that has not been discussed in committee, this is something I have seen happening often both as a delegate myself and a chair.

Creative solutions often have to be omitted because they haven't been discussed in the committee before and once the opportunity for discussing a solution/issue/topic is over (as in the last committee session before presenting the draft resolution) there's nothing u can do then.

Besides this, keep yourself motivated. Do not lose hope and give up, on that note neither should you let overconfidence get the best of you.

Now for the actual conclusion.

Conclusion

Reframing narcotics policy requires delegates to reject false binaries. The choice is not between being soft on drugs and being indifferent to human suffering, nor between public health and public safety. A credible policy can recognize addiction as a medical condition while still treating organized trafficking as serious crime. It can protect communities without criminalizing illness. It can uphold the drug control conventions while respecting the ICCPR, due process and the right to health.

The central finding of this paper is that drug policy becomes more legitimate when it distinguishes people, behaviours and harms. Personal use and dependence require health-led responses. Low-level, coerced or survival-based offending requires proportionate sanctions and

diversion. Organized criminal activity requires targeted enforcement, financial investigation and international cooperation. Capital punishment for drug offences should be replaced by proportionate sentencing because it is irreversible, difficult to reconcile with the most serious crimes threshold and vulnerable to discriminatory application. For UNCND, the strongest reframing strategy is a balanced resolution on treatment, harm reduction, proportionality, abolition or moratorium of the death penalty for drug offences, and stronger action against transnational organized crime.

Delegates should also remember that effective resolutions are not only declarations of principle. They must include implementation language: capacity-building for treatment systems, technical assistance for data collection, safeguards against arbitrary detention, training for police and prosecutors, and cooperation against trafficking networks. A humane drug policy does not become weaker because it is evidence-based; it becomes stronger because it directs the harshest tools of the State toward the most harmful conduct while protecting people whose primary need is care. The best reframing of narcotics policy is therefore both realistic and principled: reduce demand through health, reduce supply through focused justice, and reduce punishment that violates dignity without improving safety.

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All the very best and Good Luck!

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